

[120]LIVORA — HEALTHCARE PLATFORM

Business Model Report

Bangladesh Healthcare Ecosystem

Doctor-First Growth Strategy • Patient Network Expansion • Hospital Ecosystem Integration

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1 Market Context & Platform Overview

1.1 The Bangladesh Healthcare Problem

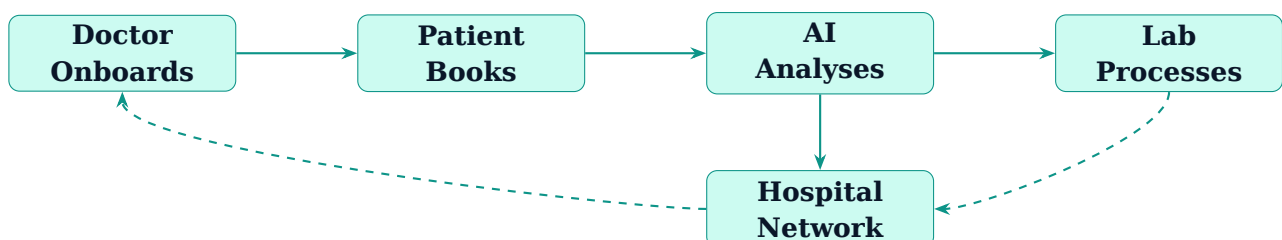
Bangladesh sits at a critical inflection point. With a population of 170 million and a doctor-to-patient ratio of **1:1,400** (World Bank), the healthcare system is structurally fragmented, analogue, and inaccessible to most citizens — especially outside Dhaka and Chittagong. Key systemic failures include:

- **Fragmented patient records:** A specialist in Mirpur sees a patient for the fourth time and still asks: *“Any previous conditions?”* There is no shared digital record.
- **Medication non-adherence:** Over 50% of patients in Bangladesh abandon prescribed medications within 30 days, resulting in preventable hospital admissions that cost the health system billions of takas annually.
- **Administrative overload:** Physicians spend an estimated 40% of their clinical time on clerical work — handwritten prescriptions, scheduling, and billing.
- **Poor care coordination:** A cardiologist at BIRDEM and a nephrologist at Square Hospital managing the same patient have no shared channel, no shared record, and no AI-assisted handoff.
- **Geographic inequity:** 65% of specialist doctors are concentrated in Dhaka, while 62% of the population lives in rural and semi-urban areas.

Market Insight: Bangladesh’s healthcare market is currently valued at approximately **₹16,000 Crore** (\$1.5B) and is projected to reach **₹25,000 Crore** by 2030. The global digital health market will reach \$573B by 2030 at a 20%+ CAGR. No dominant AI-first health platform exists in Bangladesh — the shelf is empty.

1.2 Platform Summary: Livora

Livora is a unified, AI-powered healthcare platform connecting doctors, patients, labs, and hospitals in a single ecosystem. The tagline: *“Your Healthcare Travels With You.”*



Core value propositions by stakeholder:

For Doctors	For Patients	For Hospitals
Digital Clinical Vault	Permanent Health Vault	Referral Network
AI Consultation Support	Instant History Access	Outcome Analytics
Voice-to-Prescription	AI Healthcare Guidance	Staff Credentialing
Clinical Safety Alerts	Faster Access to Care	Bed & OT Management
MDT Collaboration	Emergency Routing	API / EMR Integration
Practice Analytics	Telemedicine Access	Compliance Dashboard

2 Go-to-Market Roadmap

The strategy follows a deliberate **three-phase sequencing**: build supply (doctors) before demand (patients), then monetise both before adding the enterprise layer (hospitals). This mirrors the successful playbook of Practo in India and Zocdoc in the US, but adapted for Bangladesh’s lower average revenue per user and mobile-first usage patterns.

2.1 Phase 1 — Doctor Network (Q1-Q4 2026)

Phase 1 Targets

500 doctors onboarded · **Dhaka + Chittagong** initial cities · Free tier only · MVP product

Tactics:

- Direct outreach via Bangladesh Medical & Dental Council (BMDC) registered practitioners
- University hospital partnerships (DMCH, Chittagong Medical College Hospital)
- Referral incentive: doctors earn ₳500 credit per verified colleague onboarded
- WhatsApp-based onboarding for low-friction entry
- Free scheduling, basic telemedicine, and digital prescription tools — no payment required

2.2 Phase 2 — Patient Marketplace (Q1-Q3 2027)

Phase 2 Targets**2,500 doctors · 120,000 patients · Paid tiers launch · 3 cities**

Once 500+ verified doctors are active, patient acquisition becomes credible. Each doctor brings an average of 50–150 patients through referral and word-of-mouth.

Tactics:

- Facebook and YouTube advertising (dominant channels in Bangladesh)
- Partnership with bKash/Nagad for seamless BDT subscription payments
- Telemedicine marketing during peak seasons (flu, dengue, ramadan health)
- University campus health campaigns (large youth demographic)

2.3 Phase 3 — Hospital Ecosystem (2028-2029)

Phase 3 Targets**10,000+ doctors · 600K+ patients · 15 hospitals · National coverage**

- Direct B2B sales to private hospital chains (Square, United, Popular, Labaid, Ibn Sina)
- Pilot program with government hospitals under Digital Bangladesh / Smart Bangladesh 2041 mandate
- Referral data and aggregated analytics as primary hospital value proposition
- Enterprise integration with existing HMIS systems

3 Pricing Tiers

All prices shown in BDT (Bangladeshi Taka) and USD equivalent at approximately ₳110/\$1.

3.1 Doctor Pricing

Feature	Free	Professional	Specialist	Clinic Hub	Enterprise
Monthly Price	₹0	₹3,500	₹5,500	₹12,000	Custom
USD equiv.	\$0	\$32	\$50	\$109	—
Appointments/r	10	Unlimited	Unlimited	Unlimited	Unlimited
Telehealth sessions	2	Unlimited	Unlimited	Unlimited	Unlimited
Digital Prescription	Basic	QR-verified	QR-verified	QR-verified	Custom
AI Clinical Support	×	Basic	Advanced	Advanced	Full
Patient Health Vault	×				
Analytics Dashboard	×				
Search Priority	Standard	High	Top	Promoted	Sponsored
Team Members	1	1	1	Up to 10	Unlimited
Hospital Network	×	×			
EHR Integration	×	×	×		
Dedicated Support	Email	Priority	Priority	Account Mgr	SLA

3.2 Patient Pricing

Feature	Free	Care+ (Personal)	Family Plan
Monthly Price	₹0	₹499/mo	₹999/mo
USD equiv.	\$0	\$4.50	\$9.09
Doctor Search & Browse			
Appointment Booking	Limited	Unlimited	Unlimited (5 members)
Telehealth	1/month	Unlimited	Unlimited
Digital Prescriptions	×		
Health Records Vault	×		Shared family vault
AI Health Guidance	Basic	Full	Full
Priority Slots	×		
Emergency Routing	×	×	
Medication Reminders	×		

Note: Bangladesh’s monthly mobile internet ARPU is approx. ₹350. Pricing is calibrated to be below that threshold for entry tier and at 1.4x for premium — benchmarked against bKash financial services pricing patterns.

3.3 Hospital & Institutional Pricing

Feature	Clinic Hub	Hospital Pro	Enterprise
Annual Price	₹3,96,000	₹15,84,000	Negotiated
<i>USD equiv.</i>	\$3,600/yr	\$14,400/yr	Custom
Departments	Up to 3	Unlimited	Unlimited
Doctor Profiles	Up to 25	Unlimited	Unlimited
Referral Network			
Outcome Analytics	Basic	Advanced	Custom BI
EMR/HMIS Integration	×		
Staff Credentialing	×		
Bed / OT Management	×		
White-label Option	×	×	
SLA Guarantee	×	99.5%	99.9%
Data Residency	Shared	Bangladesh	Dedicated

Transaction Fee Model (Additional Revenue): For appointments booked through the platform, Livora charges a 5–8% booking facilitation fee on the consultation value. For telemedicine consultations, a flat ₹50 platform fee applies per session (waived for subscribed doctors). This mirrors Practo’s model which earns 5–12% per transaction.

4 Customer Acquisition Strategy

4.1 The Doctor-First Flywheel

The core insight is that **supply creates demand** in healthcare marketplaces. Patients follow doctors — not platforms. Every verified, active doctor on Livora becomes a distribution node.

4.2 Acquisition Cost Benchmarks (Bangladesh-adjusted)

Global healthcare platform CAC data suggests doctor onboarding costs \$15–\$80 per doctor via digital outreach. For Bangladesh, field-based outreach through medical rep networks reduces this significantly. Patient acquisition via organic Facebook/YouTube is far cheaper than paid search in western markets.

Segment	CAC (৳)	CAC (USD)	Annual LTV (৳)	LTV:CAC Ratio
Doctor (Free→Paid)	৳1,500	\$14	৳42,000	28:1
Patient (Free)	৳250	\$2.30	৳2,400	9.6:1
Patient (Paid)	৳500	\$4.55	৳5,988	12:1
Hospital (B2B Sales)	৳60,000	\$545	৳396,000	6.6:1
Blended Average	—	—	—	~12:1

Target LTV:CAC ratio benchmark: 3:1 minimum (industry standard). Livora’s projected ratios exceed this threshold at all segments.

4.3 Channel Mix

Channel	Target Segment	Est. Cost	Priority
Field Sales / Medical Reps	Doctors (city)	৳1,200/doc	P1
Facebook / Instagram Ads	Patients	৳200–500/pt	P1
WhatsApp & Referral	Doctors & Patients	৳50–150	P1
YouTube Health Content	Patients (rural)	৳100–300	P2
University Med Camps	Doctors (BMDC)	৳500/event	P2
B2B Direct Sales	Hospitals	৳50K/deal	P2
Google / SEO	All	Organic	P3
DGDA / Govt Partnerships	Hospitals + Govt	Grant-funded	P3

5 Financial Projections (2026-2029)

5.1 Assumptions

- Exchange rate: $\text{₳}110 = \$1$ USD (stable assumption)
- Doctor paid conversion rate: 5% in Y1, ramping to 35% by Y3
- Patient paid conversion: 3% in Y1, ramping to 10% by Y3
- Average doctor blended ARPU: $\text{₳}3,800/\text{month}$ (Professional + Specialist + Clinic mix)
- Average patient ARPU: $\text{₳}320/\text{month}$ (Care+ and Family blend)
- Hospital annual contract value: $\text{₳}8,00,000$ avg (blend of Clinic Hub and Pro)
- Transaction fee revenue: 6% avg of gross booking value
- Churn: 5% monthly doctor, 8% monthly patient (conservative)
- Sales cycle: Hospitals — 3 to 6 months

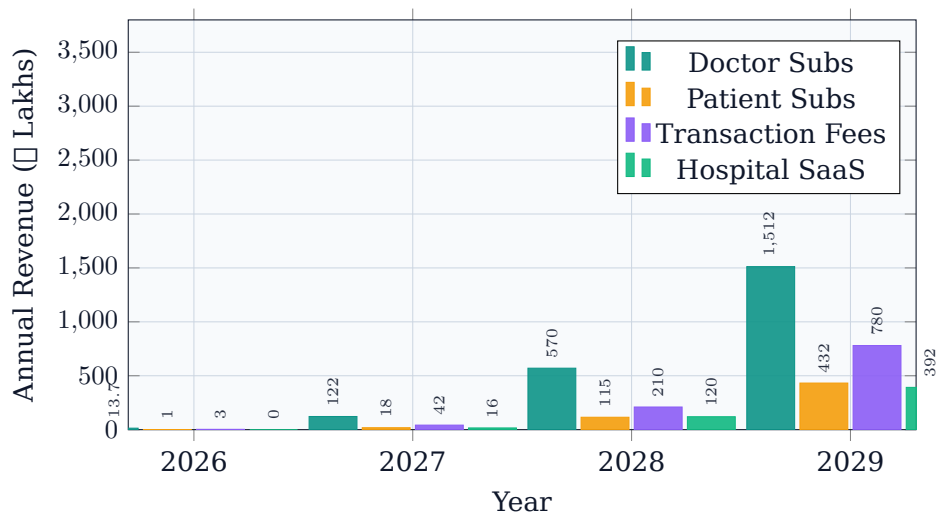
5.2 Customer Base Projections

Metric	Q4 2026	Q4 2027	Q2 2028	Q4 2028	Q4 2029
Doctors (Free)	420	2,100	4,000	6,000	9,500
Doctors (Paid)	80	600	1,800	3,500	6,000
Patients (Free)	7,000	90,000	250,000	480,000	900,000
Patients (Paid)	500	8,000	30,000	80,000	180,000
Hospitals	0	2	6	15	28
Total Registered	7,500	100,702	285,806	569,515	1,089,528

5.3 Annual Revenue Projection

Revenue Stream	2026 (₳ L)	2027 (₳ L)	2028 (₳ L)	2029 (₳ L)
Doctor Subscriptions	13.7	122	570	1,512
Patient Subscriptions	1.0	18	115	432
Transaction / Booking Fees	3.0	42	210	780
Hospital SaaS Contracts	0	16	120	392
Advertising / Sponsored	0.5	8	60	228
Total Revenue	₳18.2 L	₳206 L	₳1,075 L	₳3,344 L
<i>USD equivalent</i>	<i>\$165K</i>	<i>\$1.87M</i>	<i>\$9.77M</i>	<i>\$30.4M</i>

Note: ₳ L = Bangladeshi Taka in Lakhs (1 Lakh = ₳1,00,000). Revenue figures shown as cumulative annual total.



5.4 Revenue Mix Evolution

Revenue Stream	2026	2027	2028	2029
Doctor Subscriptions	75%	59%	53%	45%
Patient Subscriptions	5%	9%	11%	13%
Transaction Fees	17%	20%	20%	23%
Hospital SaaS	0%	8%	11%	12%
Advertising	3%	4%	6%	7%

6 Cost Structure & Operating Expenditure

6.1 Total Investment Required

To execute this plan through to break-even, Livora requires an initial **Seed/Pre-Series A investment of ₹2.75 Crore (\$2.5M USD)**, to be deployed over 18 months.

Investment Use	Amount (₹ Cr)	Amount (USD)	Share
Product & Technology	₹0.88 Cr	\$800K	32%
Go-to-Market & Sales	₹0.55 Cr	\$500K	20%
Team & Operations (18mo)	₹0.77 Cr	\$700K	28%
Infrastructure & Compliance	₹0.28 Cr	\$250K	10%
Working Capital Buffer	₹0.28 Cr	\$250K	10%
Total Seed Round	₹2.75 Cr	\$2.5M	100%

6.2 Annual Operating Cost Breakdown

Cost Category	2026 (₹L)	2027 (₹L)	2028 (₹L)	2029 (₹L)
Engineering & Product	46.2	105.6	231	418
Sales & Marketing	19.8	71.5	154	275
Cloud & Infrastructure	6.6	19.8	52.8	107.8
Customer Support	4.4	15.4	35.2	68.2
G&A / Legal / Compliance	9.9	24.2	44	74.8
COGS (hosting, bandwidth)	5.5	22	77	198
Total OpEx	₹92.4 L	₹258.5 L	₹594 L	₹1,141.8 L
<i>USD equivalent</i>	<i>\$840K</i>	<i>\$2.35M</i>	<i>\$5.40M</i>	<i>\$10.38M</i>

6.3 Headcount Plan

Department	2026	2027	2028	2029
Engineering	5	12	22	35
Product	2	4	7	10
Sales (Field + B2B)	3	8	16	28
Marketing	2	4	7	10
Customer Success	2	5	10	18
G&A / Finance / Legal	2	3	5	7
Total FTE	16	36	67	108

Average Bangladeshi tech salary assumption: ₹50,000–₹1,20,000/month depending on seniority. Engineering senior avg: ₹1,00,000/month. Sales reps avg: ₹40,000 + commission.

7 Break-Even Analysis

7.1 Break-Even Calculation

The break-even point is reached when monthly revenue equals monthly total costs. Based on our model:

Break-Even: Q1 2028

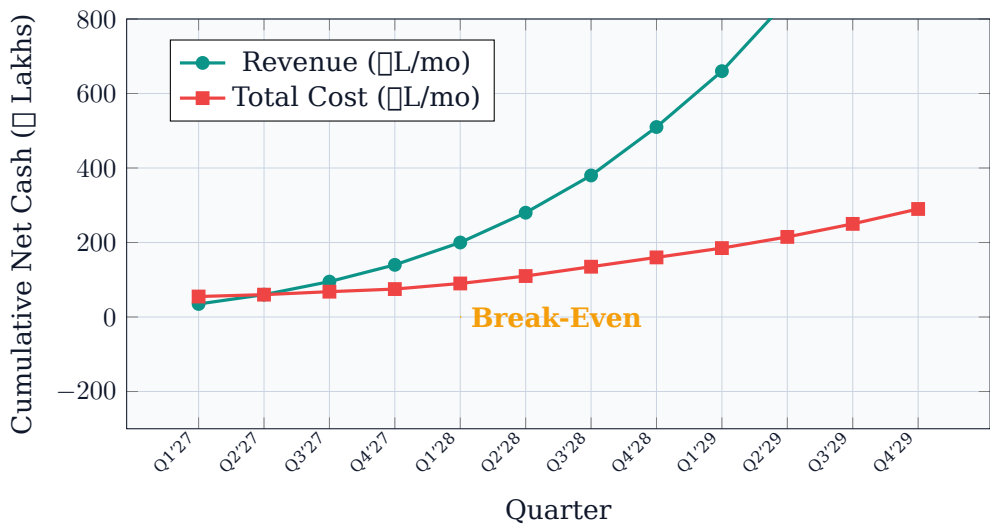
Required Monthly Revenue:	₹45 Lakhs (\$41K)
Required Active Paying Doctors:	~1,200 paid doctors
Required Active Paying Patients:	~18,000 paid patients
Hospital Contracts Required:	~4 hospitals signed
Total Monthly Costs at That Point:	₹44.5 Lakhs

7.2 Break-Even Sensitivity Analysis

The following table shows how break-even shifts based on doctor paid conversion rate — the single most critical variable in the model:

Doctor Paid Conv. Rate	Break-Even Quarter	Investment Needed	Risk Level
10%	Q3 2027	₹2.2 Cr	Low
20% (Base)	Q1 2028	₹2.75 Cr	Medium
30%	Q4 2028	₹3.5 Cr	Medium
5% (Bear)	Q2 2029	₹5 Cr	High

7.3 Cash Flow Timeline



8 Path to Profitability

8.1 Full P&L Summary

P&L Line Item	2026 (₹L)	2027 (₹L)	2028 (₹L)	2029 (₹L)
Gross Revenue	18.2	206	1,075	3,344
Cost of Revenue (COGS)	(5.5)	(22)	(77)	(198)
Gross Profit	12.7	184	998	3,146
<i>Gross Margin</i>	70%	89%	93%	94%
R&D Spend	(46.2)	(105.6)	(231)	(418)
Sales & Marketing	(19.8)	(71.5)	(154)	(275)
General & Admin	(20.9)	(59.4)	(132)	(250)
EBITDA	(74.2)	(52.5)	481	2,203
<i>EBITDA Margin</i>	(408%)	(25%)	45%	66%
Cumulative Net Cash	(74.2)	(126.7)	354.3	2,557
<i>USD EBITDA equiv.</i>	(\$675K)	(\$477K)	\$4.37M	\$20M

Key Milestone: Cumulative cash turns positive in **mid-2028**, approximately 24 months after launch of paid tiers. By end of 2029, the platform generates approximately **₹2,557 Lakhs (~\$23M) in cumulative surplus** — a full return on initial investment and the foundation for Series B fundraising.

8.2 Unit Economics at Scale (2029)

Metric	Value (₹)	Value (USD)
Revenue per Doctor (paid, annual)	₹46,200	\$420
Revenue per Patient (paid, annual)	₹3,840	\$35
Revenue per Hospital (annual avg)	₹7,93,000	\$7,209
Gross Margin	94%	—
Net Revenue Retention (doctors)	115%+	—
Payback Period (doctor)	4 months	—
Payback Period (hospital)	2 months	—

9 Competitive Positioning

Dimension	Livora	Practo	Zocdoc	Doctor.co	Shasthya Seba*
Doctor Free Tier			×		
Hospital SaaS		Partial	×	Partial	×
Bangladesh Market	Primary	Secondary	×	×	Yes
BDT Payment (bKash)		×	×	×	Partial
AI Clinical Support			×	×	×
Doctor Price/mo	□0-12K	□900-4K	\$250+	\$275+	□0-2K
EHR Integration			Limited		×
Offline-capable App		×	×	×	×
2024 ARR	—	\$30M	~\$200M	\$50M+	Est. \$1M

* Shasthya Seba and similar local platforms (Doctorola, Praava Health) exist but lack AI, EHR, and hospital-tier features.

Competitive Moat: Livora’s durable advantage is **local market depth** — BDT payments via bKash/Nagad, Bangla UI/UX, offline-lite mode for rural connectivity gaps, BMDC-verified credentials, and integration with Bangladesh’s DHIS2 national health data system. No international competitor can replicate this in 12 months.

10 Risk Framework & Mitigation

Risk	Impact	Likelihood	Mitigation
Low doctor adoption	HIGH	Medium	Free tier + referral incentive + field sales
Patient data privacy breach	HIGH	Low	DGDA compliance, ISO 27001, encrypted vault
Hospital long sales cycles	MED	High	Clinic-Hub entry tier; case study pilots
Internet/connectivity gaps	MED	High	Offline-lite app mode; SMS fallback
Burn rate overrun	MED	Medium	Milestone-gated hiring; 6-mo cash buffer
Regulatory tightening	LOW	Low	Local legal counsel; modular compliance arch
Currency depreciation (BDT)	MED	Medium	USD-denominated investor reporting; hedging
Competitor entry (Practo BD)	MED	Medium	18-mo first-mover window; BMDC partnerships

11 Key Performance Indicators

KPI	2026 Target	2027 Target	2028 Target
Growth Metrics			
Monthly Active Doctors	500	2,700	7,800
Doctor Paid Conversion	15%	22%	30%
Patient Monthly Growth	+35%/mo	+25%/mo	+18%/mo
Platform NPS Score	>40	>55	>65
Revenue Metrics			
Monthly Recurring Revenue	₹1.5L	₹17L	₹90L
Avg Rev / Paying Doctor	₹3,200/mo	₹3,800/mo	₹4,200/mo
Transaction Volume (₹/mo)	₹5L	₹35L	₹175L
Unit Economics			
Doctor LTV:CAC Ratio	10:1	18:1	28:1
Gross Margin	70%	88%	92%
Monthly Churn (Doctors)	<8%	<5%	<3%
Payback Period	8 months	5 months	4 months
Engagement Metrics			
Appointments/Doctor/M	10+	20+	35+
Patient Return Rate (6mo)	>45%	>58%	>68%
Telehealth Session Growth	+40%/qtr	+30%/qtr	+20%/qtr

11 Executive Summary & Investment Case

The Opportunity in One Page

Bangladesh has 170 million people, 1 doctor per 1,400 citizens, and no dominant digital health platform. Livora enters with a doctor-first, freemium-to-premium strategy that mirrors the global success of Practo (India) and Zocdoc (USA) — but localised with BDT payment rails, Bangla UX, BMDC verification, and an offline-capable architecture for rural connectivity.

₹2.75 Cr → Q1 2028 → ₹3,344 L ARR
Seed Investment Break-Even 2029 Revenue

Break-even in 18-20 months from start of paid tiers (Q1 2028)

₹2,557 Lakhs cumulative surplus by end-2029 — full seed return + surplus

1M+ registered users by 2029 across doctors, patients, and hospitals

66% EBITDA margin at scale — SaaS-grade economics

Network effect moat: every doctor brings 80+ patients; every hospital brings 25+ doctors

- No dominant competitor in Bangladesh — first-mover window is open for 18 months
- Smart Bangladesh 2041 national mandate aligns government incentives with platform growth

This report was prepared by Team MAKAL (Livora), University of Chittagong · June 2026

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